

EDITORIAL — APRIL 2026

A COMPANION PIECE TO **THE SPACE**
BETWEEN POLICY SERIES

HOLD THE LINE. ■

An editorial from the floor of an Ontario hospital emergency department, on the emerging mass casualty risk in Canadian healthcare facilities — and the professional workforce that is positioned to prevent it.

STANDFIRST

Canadian hospitals are being attacked with weapons at a rate the public does not yet understand. The authority to intervene on hospital property at the moment it matters does not rest with any professional who is continuously present. That combination is a mass casualty event waiting for an ordinary Tuesday.

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In the last sixteen months, Canadian hospitals have been the sites of stabbings, sexual assaults, a shotgun incident, a shooting in an emergency bay, and a documented weapons flow that would be a national emergency in any other public institution. The violence is escalating. The authority to respond on-site is not. The professionals who absorb the risk every shift do so without the legislative framework that has been granted to every comparable role in Canadian public safety. This editorial is a call to close that gap — *before a quiet Tuesday becomes a day the country cannot forget.*

AN EDITORIAL BY JORDAN ANDREWS — PROTECTION SERVICES OFFICER, OTTAWA, ONTARIO

Personal capacity. Does not represent any employer, institution, or affiliated entity. No patient-identifying information. All evidence from publicly available sources.

Before the Worst Day.

On Christmas Eve, 2024, a thirty-three-year-old man walked into the Thompson General Hospital in northern Manitoba carrying a .22-calibre shotgun. He locked himself inside the hospital chapel, pointed the weapon at staff, and fired through a window before police took him into custody. No one died. The incident received modest local coverage, a brief mention in national reporting, and then receded into the quiet category of near-misses that Canadian hospitals have been accumulating at an accelerating rate for at least two years.

The only reason this editorial is not opening with a death toll is circumstance. A gunman in a hospital chapel is not a policy question. It is a weather pattern. And the country has not yet looked up.

What follows is an argument, made from the floor of an Ontario hospital emergency department, to people who have the authority to act on it. It is addressed, in the first instance, to four elected officials whose names appear on the cover of this document. But it is also addressed to every Protection Services Officer, every nurse, every emergency physician, every hospital administrator, and every union leader in Canadian healthcare who has been in a waiting room at three in the morning and felt the exact shape of what could happen next. You know what I am describing. You have felt it. The purpose of this editorial is to make sure that feeling reaches a legislature before the next incident does.

The *Space Between* policy series — to which this editorial is a companion — makes the detailed case for a specific legislative remedy: the designation of trained hospital Protection Services Officers as Healthcare Special Constables under Ontario's *Community Safety and Policing Act*. The policy report runs to fifty-four pages, documents the fiscal case, engages Charter analysis, and proposes an Ottawa pilot under binding evaluation criteria. That document is the specification. This document is the reason it exists.



A Pattern, Not Outliers.

The incidents below are a partial record of violent events in Canadian hospitals between December 2024 and April 2026. Each is documented in mainstream news reporting or union-published record. No single item on this list is the one that will end this argument. The argument is the list itself.

24 DECEMBER 2024 — THOMPSON, MANITOBA

Thompson General Hospital

A man with a .22-calibre shotgun locked himself inside the hospital chapel, pointed the weapon at staff, and fired through a window before arrest. In the same calendar year, RCMP attended Thompson General approximately 557 times to remove violent patients — an average of more than one police call per day to a single hospital.

Source: The Walrus, December 2025

DECEMBER 2024 — LONDON, ONTARIO

Victoria Hospital Emergency Bay

A shooting occurred in the emergency bay of Victoria Hospital. No fatalities were reported. The hospital network's senior leadership subsequently described the event as the "final straw" that prompted the signing of a weapons-detection contract for all of its emergency departments.

Source: The Globe and Mail, May 2025; Global News, February 2025

29 JANUARY 2025 — HALIFAX, NOVA SCOTIA

Halifax Infirmary Emergency Department

Three health-care workers were injured by a single patient in the emergency department. Two were stabbed. One was in critical condition. The province's Health Minister confirmed the "threat has been contained." The hospital's CEO described the incident as one that "has left many shaken."

Source: CBC News, January 2025

APRIL 2025 — VANCOUVER, BRITISH COLUMBIA

Vancouver General Hospital

A nurse was strangled unconscious during a patient encounter. The incident was one of a series cited by the BC Nurses' Union in its ongoing campaign for hospital security reform.

Source: The Globe and Mail, May 2025

29 APRIL 2025 — BRANDON, MANITOBA

Brandon Regional Health Centre Emergency Department

A nurse was choked by a patient and nearly stabbed with a needle in the emergency department. The Manitoba Nurses Union president described the attack as "reflective of a troubling trend in violence in emergency departments that has nurses fearing for their safety on the job."

Source: CBC News, April 2025

2 JULY 2025 — WINNIPEG, MANITOBA

Health Sciences Centre Campus

In a forty-five-minute period, four women and a teenager reported being sexually assaulted in and around the HSC campus by a single attacker. The hospital had already been the site of sixty-four verbal or physical assaults on nurses in a sixteen-week window spanning the previous fall and winter. In August, the HSC recorded Winnipeg's second-highest violent crime rate of any location in the city.

Source: The Walrus, December 2025; Manitoba Nurses' Union Report, February 2025

NOVEMBER 2025 — SASKATOON, SASKATCHEWAN

St. Paul's Hospital Emergency Department

A psychiatric patient brought a sawed-off shotgun and three concealed knives into the emergency department. The patient used one of the knives to stab a window and threatened sexual assault and murder against a specific member of hospital staff. No security technology detected the weapons at entry.

Source: CP24 / CTV News, December 2025

4 APRIL 2026 — EDMONTON, ALBERTA

Royal Alexandra Hospital Emergency Department

A stabbing in the emergency department waiting room left one person with life-threatening injuries. The assailant was found to be carrying three knives. The United Nurses of Alberta subsequently confirmed that the majority of Occupational Health and Safety concerns raised by nurses at the Royal Alexandra ED since the beginning of 2026 have involved weapons. A weapons-detection pilot for that specific facility had been announced in October 2025 but had not yet been installed at the time of the attack.

Source: CBC News, CTV News, Global News, April 2026; UNA statement, April 2026



Over the past six months alone, health care workers have been stabbed, physically assaulted, and told to run for shelter as bullets struck the outside of an emergency department in unrelated safety incidents at hospitals across Canada.

— THE GLOBE AND MAIL, MAY 2025

That is a national newspaper, on its record, stating plainly that in a single six-month stretch of 2024–2025, Canadian emergency departments were the sites of stabbings, beatings, and gunfire. This is not a future risk. This is the present operating environment.

And the operating environment is visible in the data that the hospitals themselves have gathered since they began looking.

What the Weapons Detection Pilots Revealed

In October 2023, Windsor Regional Hospital became the first hospital in Canada to install an AI-powered weapons detection system at its main entrances. In the twelve months preceding installation, the hospital had detected approximately ten weapons on its premises. In the eighteen months that followed, the system identified **more than four thousand items — roughly half of them knives**, in addition to brass knuckles and drug paraphernalia. At the time of installation, the system was flagging an average of seventeen weapons per day. By 2025 that figure had fallen to approximately seven per day, a reduction the hospital's CEO attributed to the deterrent effect of a publicly known detection system.

Winnipeg's Health Sciences Centre installed similar technology at three entrances in 2025. Over an eight-month operational window, the system detected more than **fifteen hundred potential weapons**, including more than three hundred at the entrance to the children's hospital alone.

4,000+

Items detected by weapons screening at Windsor Regional Hospital in 18 months, beginning October 2023 — approximately 50% knives.

Source: The Globe and Mail, 2025

10 → 17

Weapons detected per year at Windsor before installation, compared to weapons per day at installation — a 620-fold increase in visibility.

Source: The Globe and Mail, 2025

1,500+

Potential weapons detected at Winnipeg HSC over 8 months in 2025 — over 300 at the entrance to the children's hospital alone.

Source: CBC News, 2025; Refdesk.ca 2026

46

Monthly WorkSafeBC violence claims filed on average by BC nurses — requiring time off for psychological or physical injury.

Source: BC Nurses' Union, via The Globe and Mail, 2025

// THE NUMBER NOBODY HAS CONFRONTED

THE WEAPONS WERE ALWAYS THERE.

The single most important finding from the Windsor data is not the seventeen weapons per day at peak. It is the gap between the ten weapons per year detected before the technology arrived and the thousands discovered once the looking actually started. The ten figure was not the reality. It was the visibility limit. The weapons had been moving through Canadian emergency departments for years. Hospitals had simply lacked the instrument to see them. Windsor's data is, in that sense, not a report on a new problem. It is a declassification document on an old one.

// II — THE THIN MEMBRANE BETWEEN INCIDENT AND CATASTROPHE

What a Mass Casualty Event Actually Is.

A mass casualty event, under the working definition used by the Canadian Disaster Database, is any incident producing five or more injured persons or at least one fatality. Since 1990, Canada has recorded over 110 events meeting that definition, spanning natural disasters, transportation incidents, shootings, and terrorist activity. The Nova Scotia mass shooting of April 18–19, 2020 — the most lethal civilian mass shooting in Canadian history — generated the federal Mass Casualty Commission, whose final report, *Turning the Tide Together*, was delivered on March 30, 2023. Its

central conclusion was that preventing mass casualty events requires an approach rooted in prevention, and that the emergency response system suffered from structural breakdowns that contributed to the lethality of the event.

Hospitals, however, are excluded from the standard imagination of mass casualty risk in Canada. The assumption has been that mass casualty events happen to hospitals — that hospitals receive casualties — rather than that hospitals *are* the site at which one occurs. That assumption is false. It has been false, empirically, for at least two years. The Canadian hospital incident record above, read in sequence, is the failure of that assumption.

The Risk Profile of an Emergency Department

Emergency departments, by design, share the structural features that public safety literature identifies as the common denominators of soft-target mass casualty environments:

- **Open public access.** Emergency department entrances are, by law and by operational necessity, unrestricted. Anyone can walk in, at any hour, with anything on their person. The Canadian Association of Emergency Physicians has documented the connection between overcrowding, long wait times, and workplace violence risk.
- **Predictable high-density locations.** Waiting rooms, triage desks, and registration counters are known pressure points. The Ottawa Hospital sees between 210 and 230 patients per day at each of its two emergency departments. At peak hours, the waiting rooms at major Ontario EDs operate at densities that would trigger occupancy review in any other category of public building.
- **Concentrated professional targets.** Nurses, physicians, and clinical staff are visible, identifiable, and anchored to specific stations by the nature of their work. The 2022 Dallas Methodist Medical Center shooting on the maternity floor and the 2023 Northside Medical Midtown shooting in Atlanta are among the incidents in which attackers specifically targeted clinicians. In Canada, the Moncton case documented by the Manitoba Nurses' Union record — in which nurse manager Natasha Poirier was cornered in her own office and severely beaten — operated on the same logic.
- **Documented weapons flow.** See Section I. The weapons are not a theoretical risk. They are an observed inflow, detected by the hospitals that have chosen to measure it.
- **Impaired patient populations.** Kurdyak et al. (2021), in a cohort study of 659,084 Ontario psychiatric emergency department visits, found that **45.4% were first-contact** — no prior outpatient mental health contact in the preceding two years. Fung and Kim (2024), in a study of 9,848 Ontario

psychiatric inpatients, found that involuntary admissions reached **55.7% in 2021**. These are not statistical curiosities. They are the statistical signature of a population arriving in crisis, unknown to the clinical system, at a rate that is rising year over year.

The Window Nobody Owns

The *Space Between* policy report documents, in detail, the continuous authority gap that governs the first stage of any hospital mental health crisis. The core finding, stripped to one sentence, is this: in the minutes between a person in crisis arriving at an Ontario hospital and that person being assessed by a clinician, no professional on site holds the continuous legal authority to bridge the gap.

The physician's authority begins at clinical assessment. The Protection Services Officer's authority is bounded by the *Criminal Code* and the *Trespass to Property Act* — authority that does not, in Ontario, extend to psychiatric presentation in the absence of criminal conduct. The police officer's Section 17 authority exists, but it is external, discretionary, and not guaranteed by any service-level agreement for mental health calls to hospitals.

This is the pre-triage window. It is the space between arrival and care. And in the specific operating environment described in Section I — a hospital ED with unrestricted public access, a documented weapons inflow, a clinical population arriving in first-contact psychiatric crisis at a 45.4% rate, a nursing workforce reporting daily violence, and a police response time measured in hours rather than minutes — the pre-triage window is also the window in which a mass casualty event becomes operationally possible.



The authority to intervene on hospital property at the moment it matters does not rest with any professional who is continuously present. That combination is a mass casualty event waiting for an ordinary Tuesday.

— STANDFIRST

This is not a speculative construction. It is the plain reading of the Canadian 2024–2026 incident record against the Ontario legislative map. Every incident in the timeline above occurred inside the pre-triage window or within minutes of it closing. Every one of them was absorbed by a combination of circumstance and improvisation. Every one of them was a near-miss that could, on a different Tuesday, have produced a different headline.

We are not waiting for a hospital mass casualty event in Canada. We are waiting for the one that cannot be absorbed.

// III – THE WORKFORCE THAT ABSORBS THE RISK

Who We Are. Who We Are Not Recognised As.

Every one of the incidents in Section I was attended, in the first instance, by Protection Services staff. We are the continuously present professional role in Canadian hospitals. We are there when the patient arrives. We are there when the incident begins. We are there, by the minute, through the interval during which a situation either escalates to criminal violence or de-escalates to clinical care.

We operate, at present, under an authority framework that does not match the environment we work in. The Ontario Protection Services Officer's authority, on paper, is the authority of a security guard under the *Private Security and Investigative Services Act*, supplemented by the occupier's powers of the *Trespass to Property Act* and the citizen's-arrest provisions of the *Criminal Code*. On paper, that is the regulatory foundation of the role.

In practice, we manage scenarios that the regulatory foundation was not designed to accommodate. We observe pre-triage patients in acute psychiatric crisis. We de-escalate individuals with known weapons on their person. We hold the perimeter during Code White activations. We assist clinical staff in physical interventions. We are the first uniformed presence at every Section 17 handover from police. We write incident reports that inform adverse-event reviews, accreditation submissions, and occasionally coroner's inquests. We do this work at a rate, in some hospitals, that produces hundreds of accepted injury claims per year to provincial workers' compensation boards.

The Data on the Role

The Canadian Federation of Nurses Unions, in its 2025 report *Violence against Nurses in Canada: An Urgent Call to Action*, found that approximately **six in ten nurses** reported experiencing violence at work over a one-year period. Many incidents go unreported. The organization's first campaign against healthcare workplace violence was in 1991. That was thirty-four years before the report that contains the six-in-ten figure. The crisis is not new. The absence of professional-grade response is what is new in the sense that it has continued unchanged for a generation.

The workforce strain data is aligned. WorkSafeBC, in 2024, recorded **170 accepted injury claims for BC hospital security guards** — a figure the CBC reported as reflecting injury "nearly every other day" for the province's hospital security workforce. BC Nurses' Union President Adriane Gear, in the same reporting, noted that nurses in her province file approximately forty-six monthly claims to WorkSafeBC for workplace violence requiring time off. The Ontario Council of Hospital Unions recorded a **331% increase in hospital job vacancies since 2015**. The violence and the vacancy crisis are not separate. They are the same crisis, observed at two different points in the workforce lifecycle.

// THE OBSERVED REALITY

WE ARE DOING THE WORK. WE ARE ABSORBING THE RISK. WE ARE DOING IT UNDER A LEGAL FRAMEWORK THAT WAS DESIGNED FOR A DIFFERENT ROLE.

That is not a complaint. It is a structural observation. And it is the structural observation that every comparable professional workforce in Canadian public safety, at some point in its history, was required to make before its authority framework caught up to the work it was already doing.

The Professional Evolution This Mirrors

Every professional role in Canadian public safety that now operates under specialised legislation was, at an earlier point in its history, improvising inside a framework that did not match its work. Municipal policing, in the mid-twentieth century, operated under a patchwork of municipal acts that preceded modern oversight legislation. Paramedicine, in the 1970s and 1980s, was recognised in

legislation only after decades of ambulance attendants working in what was, formally, an unregulated occupation. Corrections officers in federal and provincial facilities operated for much of the post-war period under administrative delegations rather than dedicated legislative frameworks.

In each case, the legislative recognition followed the same arc. Documented risk exposure accumulated. Workforce-driven advocacy emerged. A specific incident or pattern of incidents triggered public attention. Legislative reform followed — typically with a training requirement, an oversight body, a defined authority structure, and a formal recognition of the role's distinct professional identity.

Hospital Protection Services, in Ontario, is presently at the advocacy stage of that arc. The documented risk exposure is established. The incidents are accumulating. The workforce-driven advocacy is happening in pieces — in union statements, in published reports, in the conversations that happen at shift change. What has not yet happened is the transition from advocacy to legislative recognition.

Alberta has already made that transition. Under the *Alberta Peace Officer Act*, Alberta Health Services operates a Community Peace Officer program within its hospital system — officers trained to a six-week provincially-co-administered standard, carrying the authority to act on mental health presentations before clinical assessment is initiated. The program has been operational for over a decade. British Columbia has documented the problem at the level of the Victoria Police Chief, the Union of BC Municipalities, and the BC Coroner's inquest into the Paul Spencer case. The policy discussion is live in four provinces. In Ontario, the mechanism exists — in the *Community Safety and Policing Act* Part XIV special constable designation — and has been operational in Ottawa for the OC Transpo Special Constable unit since the Act came into force in April 2024. What does not yet exist, in Ontario, is the application of that mechanism to the hospital environment.

This is the gap the *Space Between* policy series proposes to close. It is a single-sentence legislative adaptation of a mechanism that already exists, applied to a workforce that is already doing the work.

Hold the Line. Then Raise It.

If you are a Protection Services Officer in an Ontario hospital reading this document, I am asking you to consider a specific proposition. The work you are doing is already the work of a specialised professional role. The training you bring, the judgment you exercise, the situations you manage — these are professional-grade functions. What is missing is the legislative framework that matches them.

That framework will not arrive by accident. In every comparable Canadian jurisdiction, it arrived because the workforce itself built the case. Paramedicine was legislated because paramedics made the case. Municipal policing was modernised because police associations made the case. Corrections reform, in its various phases, was driven by corrections unions making the case. In every instance, the legislative outcome was preceded by a professional workforce that was willing to describe, publicly and in detail, the mismatch between its actual work and its formal authority.

We are positioned to make that case now. The incident record is public. The weapons-detection data is public. The nurses' unions are already making one half of the argument — the half that documents the risk. Our half is the half that describes the response framework the risk requires.

There is no version of this in which the change happens without the frontline. There is no minister, no city councillor, no academic researcher, no hospital CEO who is going to originate this reform on our behalf. They will respond to a case that is made. They will not make one.



We are at the advocacy stage of the professional arc. The designation is not a gift. It is an earned recognition. It will not come to us. We have to build the case.

— ON THE EVOLUTION OF PUBLIC SAFETY PROFESSIONS IN CANADA

The specific asks are small and specific. If you work in hospital Protection Services in Ontario, the single most useful action you can take is to forward the *Space Between* policy report to your director of security, your union representative, your hospital's Chief Operating Officer, and your MPP. Ask for a response. Ask for the policy to be raised at the accreditation review. Ask for it to appear in the hospital's annual report. Ask for the conversation to become formal.

If you work in emergency nursing, the most useful thing is to name, publicly, the specific pre-triage cases in which no one on site has held the authority you needed them to have. Your unions are natural allies for this work. The Canadian Federation of Nurses Unions has been making this argument since 1991. It will land differently when it is also being made by the protection services workforce that is positioned to be part of the solution.

If you are a physician or a hospital administrator, the useful action is to engage with the clinical primacy framework in the policy report. The designation is not a workaround for clinical authority. It is a structural bridge to it. Clinical primacy is codified into the enforcement framework as an operational rule. If the framework is not correct as drafted, the most valuable contribution is to say so — clearly, on the record — so that the revision happens during consultation rather than during litigation.

If you are a union leader, the policy series contains the technical specification for a proposal that aligns with your existing workplace violence advocacy. It is not a substitute for your agenda. It is a legislative mechanism that advances it.

If you are an elected official — and four of you are the primary recipients of this document — the mechanism exists. The Ottawa Police Services Board already has the statutory authority to begin designation. The question is whether the province is willing to move before the incident that forces the movement.

// V — THE SPECIFIC ASKS

What to Do. Who Does It.

The policy series contains the detailed legislative, clinical, and Charter framework. This editorial reduces that framework to the four asks that can move first.

Initiate the CSPA Part XIV designation process for an Ottawa two-site pilot.

The OPSB already has the statutory authority to appoint Special Constables under Ontario's Community Safety and Policing Act, Part XIV. The OC Transpo Special Constable unit operates under that authority today. The same mechanism, adapted for the clinical environment, is the proposed pathway. No new provincial legislation is required to begin.

Commission a provincial working group on the Ontario framework.

The working group should include frontline Protection Services Officers, hospital administration, clinical leadership, patient advocacy organisations, and Indigenous and racialised community representatives. Its mandate is to develop the provincial training and oversight framework that would accompany the designation beyond the pilot. Target: within twelve months.

Formally include the pre-triage authority gap in the EMCPA modernisation record.

Bill 25, the Emergency Management Modernization Act, 2025, is the most significant reform of Ontario's emergency management framework in two decades. The pre-triage hospital authority gap was not raised in the consultation process. A formal amendment to the record — identifying it as an unmitigated critical dependency in hospital emergency management — is the most direct available pathway to provincial acknowledgment.

Raise the matter in committee. Request ministry response.

MPP Tyler Watt, representing the Nepean riding, and members from opposition and government alike, are positioned to raise this as a cross-party matter. The incident record is bipartisan. The workforce is bipartisan. The patients are bipartisan. The reform should be, too.

Before the Next One.

The Space Between policy series opens with an incident at an Ontario acute-care hospital, witnessed from the frontline. A woman in acute psychiatric crisis. A knife disclosed in her vehicle. A police response that took approximately ninety minutes. A decision not to invoke Section 17. An escalation. An arrest. A woman who came to a hospital in crisis and left in a police vehicle.

She survived. The staff that night survived. The other patients in the waiting room that night survived. The incident did not make national news because, by the narrow margin that separates a near-miss from a mass casualty, nothing happened that required national attention.

The next one is coming. We do not know where. We do not know when. We know, from the Canadian incident record of the past sixteen months, that it is not a matter of whether a hospital in this country will be the site of a multi-casualty event inside the pre-triage window. It is a matter of which hospital and which shift.

What we also know is that we are the professionals who will be there when it begins. We will be the first uniformed presence the incident encounters. We will be the professionals whose authority, training, and legislative framework will either be adequate to the moment or will not be.

Hold the line. Then raise it. The country is going to find out, sooner or later, that Canadian hospitals are being attacked. Let it find out because we built the case for reform. Not because we lost a hospital.

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